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Hybrid argon plasma coagulation after endoscopic mucosal resection—some caveats in the comparison with snare tip soft coagulation



We read with great interest the study by Motchum et al,¹ who analyzed the performance of hybrid argon-plasma coagulation (APC), a technique whereby APC is preceded by submucosal injection of saline through a high-pressure water jet² after EMR of large colonic lesions. The reduction of post-EMR recurrence rates through the use of APC or snare-tip soft coagulation (STSC) on the normal-appearing scar margins is well documented.^{3,4} Prior studies had also suggested that STSC may be preferable to APC.⁵

In this study, Motchum and et al¹ found lower recurrence in the first surveillance colonoscopy after hybrid APC (2.2%) than that reported for STSC (6.0%, in a recent analysis), as well as a lower percentage of serious side effects (3.6% versus 8.5%).⁶ These results are excellent and allow hypothesizing that hybrid APC may be the method of choice, potentially because of the safer ablation of deeper tissue.⁷ Notwithstanding, some caveats arise, including the small sample size, the single-arm design, and the lack of standardization of the hybrid APC technique, given that most patients (72.0%) underwent ablation of both scar surface and margins, hampering comparison with prior STSC studies. No recurrence at 6 months was observed in another recent pilot study⁸ of hybrid APC for colonic large lesions where scar surface ablation was performed in all patients, being plausible that such ablation may justify the lower recurrence rate. Another drawback is the lack of cost effectiveness analysis. In fact, although STSC does not involve additional costs, hybrid APC carries an incremental cost effectiveness ratio of \$158.7/1.0% (or \$603.0 per procedure for a 3.8% decrease in recurrence), and fixed costs of around \$50,000 for Erbejet2 and APC3 modules (prices for Portugal). Additional studies are needed to compare head-to-head standardized STSC, APC, and hybrid APC, to evaluate characteristics of the lesions that may lead to selecting 1 technique over another, and to evaluate cost effectiveness.

DISCLOSURE

Both authors disclosed no financial relationships.

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Are sex-specific benchmarks for clinically significant serrated polyp detection warranted?



To the Editor:

We read the recently published article by Anderson et al¹ and the accompanying editorial by Macaron et al² with interest. The study demonstrated that higher detection rates of clinically significant serrated polyps (CSSP-DRs) are associated with lower rates of postcolonoscopy colorectal cancer. The findings support using CSSP-DR as an independent quality measure for colonoscopy. Although target adenoma detection rates are largely agreed upon, there remains heterogeneity of serrated polyp definitions in various